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AMENDMENT PAGE

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1. LOCATION

The Operating Theatre is located within the same block as the Dental Clinic, X-Ray and the Laboratory.

2. VISION

Providing quality, effective and safe surgical services.

3. MISSION

- 3.1 To provide effective, efficient and safe perioperative and anaesthetic services.
- 3.2 To perform surgery as soon as possible to minimize the risk of infection and promote an early recovery process.
- 3.3 To improve the quality of staff service through continuous training programs.
- 3.4 Providing the latest equipment to ensure service quality is always at an optimal level.

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4. OBJECTIVE

- 4.1 To provide good quality effective and safe anesthetic services to the patient undergoing surgical procedures.
- 4.2 To provide an efficient and effective service in an optimum environment to patient undergo surgery so as to relieve suffering, save lives and achieve early recovery.
- 4.3 To ensure operation performed on the correct patient and correct procedure in every cases.
- 4.4 To provide selective intervention both in-patient and out-patient.
- 4.5 To provide patient or relatives with better understanding of the surgical or anesthesia process.
- 4.6 To provide safe, effective and efficient service in every cases or procedures.

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5. SCOPE OF SERVICES

- 5.1 Provide anesthetic service to patients undergoing surgery i.e. general anesthesia & regional anesthesia.
- 5.2 Involved in hospital networking i.e. General Surgery, Orthopedic, Pediatric, ENT and O&G, whereby surgeons from Sarawak General Hospital and UNIMAS operating their selected elective cases here
- 5.3 There is also Anesthetic Service provided by the Department of Anesthesia Sarawak General Hospital staying in the hospital quarters per weekly rotation.
- 5.4 Service is available 24 hours/day, normal office hours 8am to 5pm (Monday to Friday) with on call arrangements outside these hours (includes weekend and public holiday) for emergency operations.
- 5.5 The on-call time taken to arrive from living residence of OT staffs was LESS THAN 30 MINUTES prior from the moment being called.
- 5.6 Two Operation Theatres were available for usage, but only ONE (1) shall remained to be operatable for VISITING ELECTIVE CASES whereas ONE (1) shall be STANDBY for ANY IMPENDING EMERGENCY CASES. During scheduled specialist visiting session, the Minor OT shall be used as a recovery room before discharging the patient back to the ward.

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7.2.2 Priority is given to the pediatric cases. Parents are encouraged to accompany their children.

7.2.3 Clean cases shall be done before infected or dirty cases.

7.2.4 Emergency cases may be done between elective cases or at any other time.

7.3 Code of Dress:

7.3.1 Every personnel entering Operating Theatre shall change to proper O.T attire (**mandatory**).

7.4 Discharges:

7.4.1 All patients must be checked by the Anesthetic Specialist, Anesthetic Medical Officer or Anesthetic Assistant before transferred out from O.T.

7.4.2 A trained Nurses shall collect patient from O.T.

7.5 Cleaning, Disinfection and Sterilization (*Policies and Procedures of Infection Control, KKM*)

7.5.1 Existing regulation as stated in the:

a) Guidelines on Control of Hospital Acquired Infections KPK, Circular 2/90 Bil. (33) dlm. KKM.87 (49/5) dated 7.2._1990. Page 39-47.

b) Disinfection and Sterilization Policy and Practice, Ministry of Health 1990 (Revised) shall be adhered to.

c) Policy & Procedure on Infection Control, Quality Medical Care Section, Medical Development Division, M.O.H Malaysia. Prepared in 2009, Printed in 2010_2nd Edition (Chapter 4.7 & Chapter 6.3)

7.5.2 Cleaning and Housekeeping of premises including supply of clean linen shall be done by the Concession Company which are appointed by the

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MOH, according to schedules agreed by the hospital and concession company.

7.5.3 All O.T packs and sets shall be autoclaved by the CSSD.

7.5.4 The routine Environmental Monitoring specimen shall be performed monthly and for the Environmental Culture & Sensitivity are in quarterly basis.

7.5.5 Any outside trolley entering to the O.T, the wheels need to be disinfect with presept or Alcohol 70% prior entry.

7.5.6 Universal Precautions shall always be observed and practiced.

7.6 Maintenance and Equipment

7.6.1 The A.M.O i/c shall maintain and update inventory of all O.T equipment or assets and also to ensure that all the equipment are serviced regularly and maintain in good order.

7.6.2 Any trouble shooting or breakdown of the equipment shall be referred to the HSS.

7.7 Departmental Meeting

7.7.1 It shall be held quarterly to discuss departmental problems and ways to improving productivity, quality, efficiency and effectiveness. The meeting shall be chair either M.O. i/c, Nursing Sister i/c or Asst. M.O. i/c and attended by all O.T staffs.

7.7.2 Problems that not be solved at the departmental level shall be brought up with the Hospital Director.

7.8 Patients and Family Right.

7.8.1 Patient should be informed regarding the surgery or anesthesia in which he/she is going to undergo before signing the consent form. (*Surgical consent, anesthesia consent*)

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- 7.8.2 Patient shall be treated accordingly in respect regardless of the color, race and religion.
- 7.8.3 The Hospital Director or Specialist may signed the consent form for patients who is under 18 years old, in which there is NO parent or guardian around and in any emergency cases without relatives available.
- 7.8.4 Every patient has a right to refuse to any procedure or treatment.
- 7.8.5 Patients are advised not to bring any of their properties into the Operating Room.
- 7.8.6 Confidentiality of all patients shall be maintained.

8 WHOLE HOSPITAL POLICIES

Relevant aspects of the whole hospital policies shall be observed.

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